Sample 7-minute ISBAR: Deb

Deborah Fraser · 58-year-old woman · Community occupational therapy home visit · about 1041 spoken words (≈ 7.4 min at 140 words per minute)

I Introduction

61 words

"Hi, I'm the occupational therapy student who has been completing a home visit with Deb Fraser this morning. Deb is a 58-year-old woman with dyskinetic cerebral palsy, GMFCS level II. I need to discuss her with you urgently because she has new, progressive symptoms that I think need prompt medical review, and she's about to drive her mother to an appointment."

S Situation

118 words

"I came to assess Deb's request for grab rails in the bathroom. During the visit, she described three months of numbness and tingling in both hands, like wearing gloves, and constant neck pain that's worse when she looks up. She dropped a mug today, says it's the third this week, and fumbled her car keys.

"She also reports legs that feel stiff and heavy, four or five falls since winter, mostly when hurrying to the toilet or turning, and recent bladder accidents because she can't get to the toilet in time. She thinks it's just ageing with cerebral palsy.

"Right now, she's picking up her keys to drive her mother, Margaret, to a memory clinic appointment at eleven."

B Background

153 words

"Deb lives in Toowoomba with her 86-year-old mother, who has dementia. She retired early from her job as a library technician to care for Margaret. She walks without an aid at home, has lifelong involuntary movements of her arms, neck and face, and drives a modified automatic car with a steering spinner knob. Deb is an NDIS participant, and Margaret receives Support at Home services.

"Her GP letter four months ago described her cerebral palsy as long-standing and stable, with normal blood tests, and her NDIS plan says she manages her own personal care and Margaret's meals and medications.

"I observed her move stiffly in the bathroom and catch her toe on the shower hob.

"There are gaps: I don't know whether her GP is aware of the hand symptoms, falls or bladder changes, what medications she takes, whether she's had neck imaging, or who could care for Margaret if Deb became unwell."

A Assessment

352 words

"My main concern is that this isn't Deb's usual cerebral palsy. Cerebral palsy itself is non-progressive, so new or worsening symptoms in adulthood shouldn't just be put down to ageing. Her cluster of signs — neck pain, numbness and tingling in both hands, dropping objects, stiff, heavy legs, more falls and new bladder urgency and accidents — involves both arms, both legs and the bladder. That pattern may indicate compression of the spinal cord in

the neck. Adults with dyskinetic cerebral palsy are known to be at increased risk of this, because years of involuntary neck movements can cause early wear in the cervical spine. I can't diagnose that, but it warrants prompt medical review, because untreated compression can cause lasting loss of function.

"Although her GP described her as stable four months ago, her symptoms started about three months ago, so they may not have been reported. Her cheerfulness and independence are genuine strengths, but she's minimising symptoms, partly because she puts her mother first.

"The most immediate safety risk is driving. With numb hands that are dropping objects and fumbling keys, her ability to grip and control the spinner knob and react quickly may be reduced, putting Deb, Margaret and other road users at risk today. Under Austroads Assessing Fitness to Drive, changes in a condition that may affect driving need medical assessment, and drivers have a responsibility to report these to Transport and Main Roads. I can't make a fitness-to-drive decision, but I'm worried about today's drive.

"Her falls, especially in the bathroom with poor footing over the hob, mean an increased injury risk now, and the bladder urgency adds to her rushing to the toilet.

"There's also a carer issue. Deb is Margaret's main carer, so if Deb is unwell, injured or can't drive, Margaret's care and appointments are affected.

"The grab rails are still useful, but if the underlying cause isn't identified, rails won't solve the problem. The lift chair and NDIS questions can wait.

"So my priorities are: first, safe transport today; second, prompt medical review; third, falls risk at home; and fourth, support for Margaret."

R Recommendation

357 words

"My immediate recommendation is that I talk to Deb honestly about my concerns and strongly encourage her not to drive today. She's an adult and it's her decision, but I'd explain the specific risk. I'd offer to help arrange alternative transport, such as a taxi, a family member or Margaret's Support at Home provider, or help her call the memory clinic to explain and reschedule.

"Second, I'd like your help to contact her GP today to request a prompt appointment, ideally today or tomorrow, so the GP can examine her and consider urgent imaging and specialist referral. I'd send a written summary of what I observed. I'd also advise Deb that if she notices sudden weakness, worsening numbness or loss of bladder or bowel control, she should go to the emergency department or call triple zero.

"Third, while she's waiting, I'd recommend simple falls strategies today: sitting to shower on a shower chair, if our service can loan one, keeping a clear path to the toilet and a night light, and having her phone with her.

"For the team, I'd ask the GP to assess for a neurological cause, review her medications and bladder symptoms, and review her fitness to drive. A physiotherapist could assess her mobility and falls risk once medical causes are explored. Social work could help with carer support, including Carer Gateway and respite through Margaret's Support at Home services.

"For OT, my principal short-term assessment would be observed performance of her bathroom and kitchen routines, structured using the Westmead Home Safety Assessment, so recommendations match how her function is changing, not just the original request. Longer

term, I'd use the COPM with Deb to understand her priorities, which I expect will include caring for her mother, staying independent and keeping her licence. If her doctor recommends it, an OT driver assessment could follow.

"I'd hold off finalising grab rail placement until we know more, because her needs may change. I'd also schedule time to discuss the lift chair and her NDIS questions.

"I'd follow up by phone tomorrow to check she's seen her GP, and rebook a home visit to continue the assessment."

Coaching notes *not spoken*

The immediate priority

Deb has three months of progressive symptoms in both hands, both legs and bladder with neck pain, which in dyskinetic cerebral palsy warrants prompt medical review for possible cervical spinal cord compression. The immediate safety issue is her plan to drive today with numb, clumsy hands.

Distractors in this case

- **Grab rail request** — useful but treats the symptom, not the cause.
- **GP letter: "long-standing and stable"** — written before most symptoms started.
- **"Just getting old with CP"** — CP is non-progressive; new decline needs explaining.
- **Mother's care, lift chair brochure, NDIS questions** — real and important, but not urgent.
- **Cheerful and organised** — she is minimising symptoms to protect her caring role.

Missing information to clarify

- Whether the GP knows about the new symptoms
- Current medications
- Any previous neck imaging
- Back-up carer for Margaret
- Bowel changes, and the exact pattern of falls

Useful references

- Austroads (2022). *Assessing Fitness to Drive* for commercial and private vehicle drivers
- Carer Gateway (Australian Government)

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